

Aisha Patel

MR#: MR-38201

Physician: Dr. Abc

Date: Nov 10, 2024 · 08:10

Visit Type: Note #2

Vitals

BP: 132/82 mmHg Pulse: 80 bpm Temp: 36.9 °C SpO₂: 98 % Weight: 78 kg

CHIEF COMPLAINT

Fatigue, Excessive thirst, Blurred vision

HISTORY OF PRESENT ILLNESS

55-year-old male with known Type 2 Diabetes Mellitus presents for quarterly review. Reports increasing fatigue, polydipsia, and mild bilateral blurred vision over 3 weeks. Last HbA1c: 8.2 % (3 months ago). Home glucose: fasting 180–210 mg/dL, post-meal 240–280 mg/dL. On Metformin 500 mg BD. Compliance good. Denies polyuria or unexplained weight loss. Foot inspection reveals no active ulcer.

ALLERGIES

No known drug allergies — — (Mild)

MEDICAL HISTORY

Type 2 Diabetes Mellitus — diagnosed 2020

Dyslipidemia — on Atorvastatin 20 mg

SURGICAL HISTORY

Cataract surgery — right eye, 2022 (phacoemulsification)

FAMILY HISTORY

Mother — Type 2 DM and coronary artery disease

Sister — Type 2 DM, diagnosed at 45

SOCIAL HISTORY

Non-smoker, Non-drinker, Retired school teacher, Sedentary lifestyle — minimal daily activity

REVIEW OF SYSTEMS

Fatigue — present, worsening gradually

Polydipsia — present, significant

Polyuria — mild

Blurred vision — bilateral, intermittent

Tingling in toes — present (peripheral neuropathy symptom)

Chest pain — absent

PHYSICAL EXAMINATION

General: Overweight (BMI 29.4), no acute distress

Fundoscopy: Early background retinopathy bilateral — dot haemorrhages noted

Foot: Intact skin bilaterally; reduced monofilament sensation right > left; no ulcer

CVS: Regular rate and rhythm, no murmur

POINT OF CARE LABS**Fasting Glucose:** 188 mg/dL (Abnormal)**HbA1c (POC):** 8.4 % (Abnormal)**Urine Ketones:** Negative (Normal)

ASSESSMENT & DIAGNOSIS

E11.9 — Type 2 Diabetes Mellitus — Poorly Controlled (High)

E11.36 — Diabetic Retinopathy — Background Stage (Moderate)

E11.40 — Diabetic Peripheral Neuropathy (Low)

LAB ORDERS

HbA1c, Fasting lipid panel, Urine microalbumin / creatinine ratio, Renal function test, LFTs

PRESCRIPTIONS

Sr.	Drug	Instructions	Qty
1	Metformin 1000 mg	BD with meals (dose doubled)	60
2	Sitagliptin 100 mg	Once daily — new addition	30
3	Omega-3 Fatty Acids 1000 mg	Once daily with meal	30

IMAGING ORDERS

Fundus photography — diabetic retinopathy grading

PROCEDURE ORDERS

- Foot examination with 10 g monofilament sensory test
- Ankle-brachial index (ABI) measurement

CARE PLAN

- Diabetic diet — low carbohydrate, low GI foods
- Target HbA1c < 7 %
- Daily foot inspection at home
- Refer to diabetic educator for self-management training
- Ophthalmology referral for retinopathy management
- Review in 4 weeks for glucose response

REFERRALS

- Ophthalmology — Background diabetic retinopathy — grading and management
- Diabetic Educator — Self-management training and diet counselling
- Podiatry — Peripheral neuropathy foot care programme

PATIENT GOALS

- Bring HbA1c below 7 % within 3 months
- Daily foot inspection — check for wounds, redness, swelling
- Reduce carbohydrate intake and follow low-GI meal plan

OTHER ORDERS

- Glucometer prescribed — with lancets and test strips
- Diabetic foot care kit provided
- Medical alert bracelet recommended

VISIT SUMMARY

Poorly controlled T2DM with evidence of early microvascular complications (retinopathy, peripheral neuropathy). Medication regimen intensified. Three specialist referrals placed. Comprehensive patient education delivered. Repeat HbA1c in 3 months.

FOLLOW-UP

05 Oct 2024

Attending Physician: Dr. Aho · Facility: NovaDoc Health